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Mood Disorders

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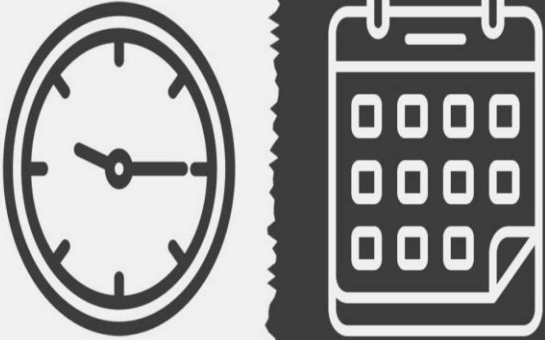


Mood Disorders

Objectives:

- Difference between mood and affect.
- Describe the symptoms, results, and risk factors of major depressive disorder
- Understand the differences between major depressive disorder and persistent depressive disorder, and identify two subtypes of depression
- Describe the symptoms and risk factors of bipolar disorder
- Describe genetic, biological, and psychological explanations of major depressive disorder
- Discuss the relationship between mood disorders and suicidal ideation, as well as factors associated with suicide

Affect	v/s	Mood
Fluctuating		Sustained
		

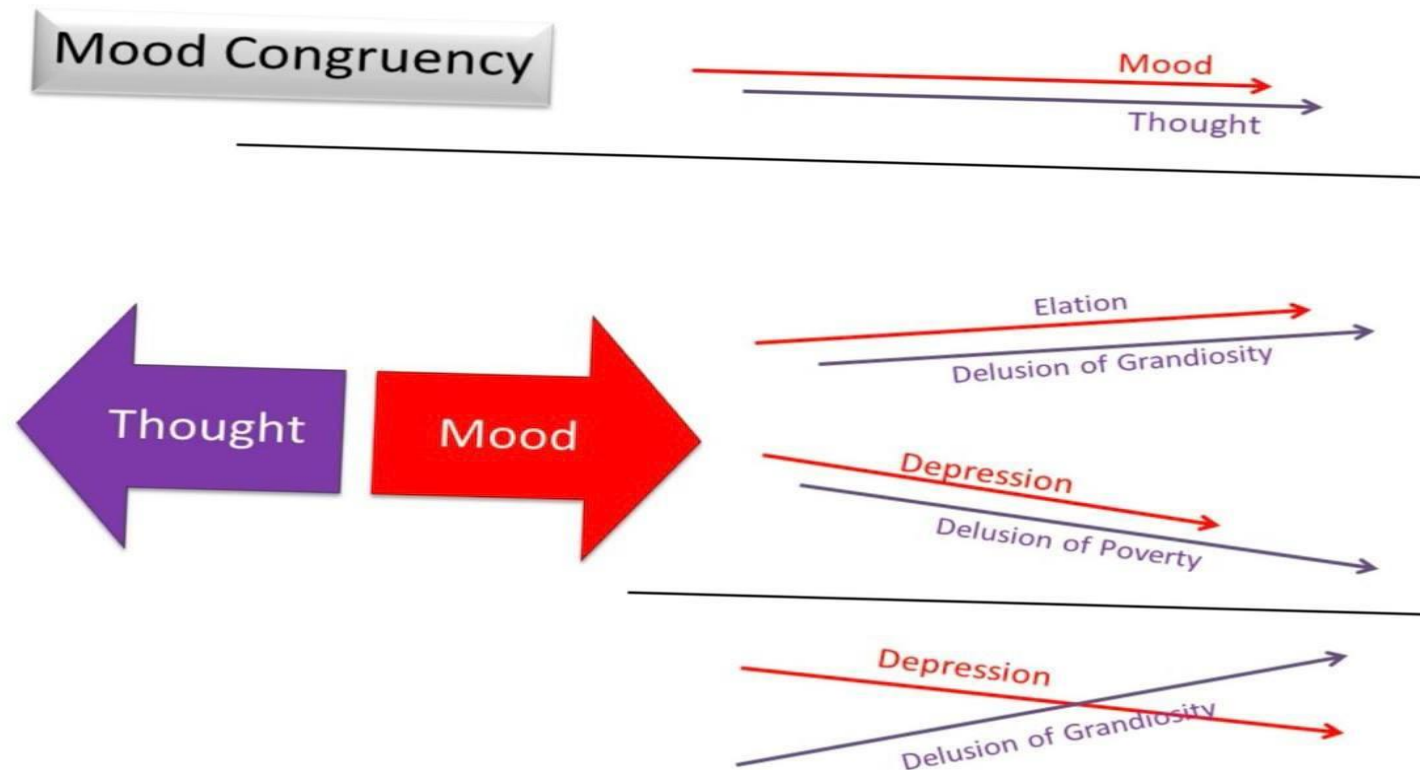
Affect	v/s	Mood
Short-lived		Can last for days or months
		

Affect	v/s	Mood
Immediate expression of emotions		Prolonged emotional state
		



What is congruency

In psychology, congruency refers to the alignment between an individual's internal experiences, feelings, and thoughts and their external expression or behavior.





Classification of Depressive Disorders

	MDD	PDD (Dysthymia)	DMDD	PMDD
Symptoms				
Mood (Note: can be depressed, irritable or angry and can fluctuate)	Depressed mood	Depressed mood	Severe recurrent temper outbursts Irritable mood	Mood swings Depressed mood Irritable or angry mood
Anhedonia	+	–	–	+
Psychomotor symptoms	Agitation or retardation	–	–	–
Anxiety/tension	–	–	–	+
Suicidal ideation	Recurrent thoughts of death or suicidal ideation, or attempt, or plan	–	–	–
Appetite changes	Significant increase or decrease in weight or appetite	Poor appetite, or overeating	–	Marked change in appetite, over-eating or specific cravings
Insomnia/hypersomnia	+	+	–	+
Low energy/fatigue	+	+	–	+
Poor Concentration	+	+	–	+
Low Self-esteem	–	+	–	–
Negative Cognitions	Worthlessness or excessive guilt	Hopelessness	–	Overwhelmed or out of control

Major depressive disorder (MDD) – Definitions and diagnosis

DSM-5-TR criteria for MDD

A.

1 Depressed mood for most of the day,
based on self-report or observation of others

or

2 Marked reduction or **loss of interest or pleasure**
in nearly all activities for most of the day

3 Significant non-dieting **weight loss** or weight gain
(>5% change in body weight)

4 **Insomnia or hypersomnia**

5 **Psychomotor agitation or retardation**
(observable by others)

6 **Fatigue / loss of energy**

7 Feeling of **worthlessness** or excessive /
inappropriate guilt

8 **Diminished cognitive function**
(reduced ability to think or concentrate)

9 Recurrent **thoughts of death** and/or suicide,
Suicide planning, or a suicide attempt

- At least five of the following symptoms have been present during the same 2-week period and represent a change from previous functioning: at least one of the symptoms is either 1) depressed mood or 2) loss of interest or pleasure

Any 5

B. Symptoms cause clinically significant distress or
impairment in social, occupational or other important
areas of functioning



C. Symptoms are not due to the direct physiological
effects of a substance (e.g., a medication) or another
medical condition (e.g., hypothyroidism)



Persistent Depressive Disorder

- Depressed mood for at least 2 years; 1 year for children/adolescents
- PLUS 2 other symptoms:
 - Poor appetite or overeating
 - Sleeping too much or too little
 - Poor self-esteem
 - Trouble concentrating or making decisions
 - Feelings of hopelessness
- Symptoms do not clear for more than 2 months at a time
- Bipolar disorders are not present



Premenstrual Dysphoric Disorder

- In most menstrual cycles during the past year, at least five of the following symptoms were present in the final week before menses and improved within a few days of menses onset:
 - Affective lability
 - Irritability
 - Depressed mood, hopelessness, or self-deprecating thoughts
 - Anxiety
 - Diminished interest in usual activities
 - Difficulty concentrating
 - Lack of energy
 - Changes in appetite, overeating, or food craving
 - Sleeping too much or too little
 - Subjective sense of being overwhelmed or out of control
 - Physical symptoms such as breast tenderness or swelling, joint or muscle pain, or bloating



Disruptive Mood Dysregulation Disorder

- Severe recurrent temper outbursts, including verbal or behavioral expressions of temper that are out of proportion in intensity or duration to the provocation.
- Temper outbursts are inconsistent with developmental level.
- The temper outbursts tend to occur at least three times per week.
- Negative mood between temper outbursts most days.
- These symptoms have been present for at least 12 months and do not clear for more than 3 months at a time.
- Temper outbursts and negative mood are present in at least two settings (at home, at school, or with peers) and are severe in at least one setting.
- Age 6 or older (or equivalent developmental level).
- Onset before age 10.
- There has never been a distinct period lasting more than 1 day during which elevated mood and at least three other manic symptoms were present.
- The behaviors do not occur exclusively during the course of major depressive disorder and are not better accounted for by another mental disorder.
- This diagnosis cannot coexist with oppositional defiant disorder, attention-deficit/hyperactivity disorder, intermittent explosive disorder, or bipolar disorder.



Bipolar Disorders

	BD I	BD II	Cyclothymia
Main Symptom Criteria (Mania)			
Elevated or irritable mood	+	Often irritable	+
Increased activity or energy	Goal-directed	+	+
Increased self-esteem	+	+	+
Decreased need for sleep	+	+	+
Pressured speech	+	+	+
Distractibility	+	+	+
Increased risk taking behaviour (especially for those with comorbid BPD)	+	+	
Main Symptom Criteria for Depressive episodes (Same as MDD)		+	
Severity and duration of episodes			
(Hypo)Mania	Mania	Hypomania**	Sub-threshold Mania
Number of Symptoms	3–4 symptoms	3–4 symptoms	≤ 3 symptoms
Duration of Episode	> 7 days	4–7 days	< 4 days
Impact on functioning	Disrupts social and occupational functioning or results in hospitalisation	Not severe enough to disrupt functioning or result in hospitalisation	Symptoms of (hypo)mania/depression cause significant distress or impairment in functioning
Depression	Depression	Depression	Sub-threshold Depression
Number of Symptoms	> 5 symptoms	> 5 symptoms	≤ 5 symptoms
Duration	2 weeks	2 weeks	< 2 weeks
Frequency of episodes	≥ 1 manic episode*	> 1 hypomanic + ≥ 1 depressive episode	Fluctuating subthreshold hypomanic and depressive symptoms for > 2 years (> 1 year for children/adolescents)



Subtypes of Major Depressive Disorder and Bipolar Disorders

Table 5.2 Specifiers of Depressive Disorders and Bipolar Disorders

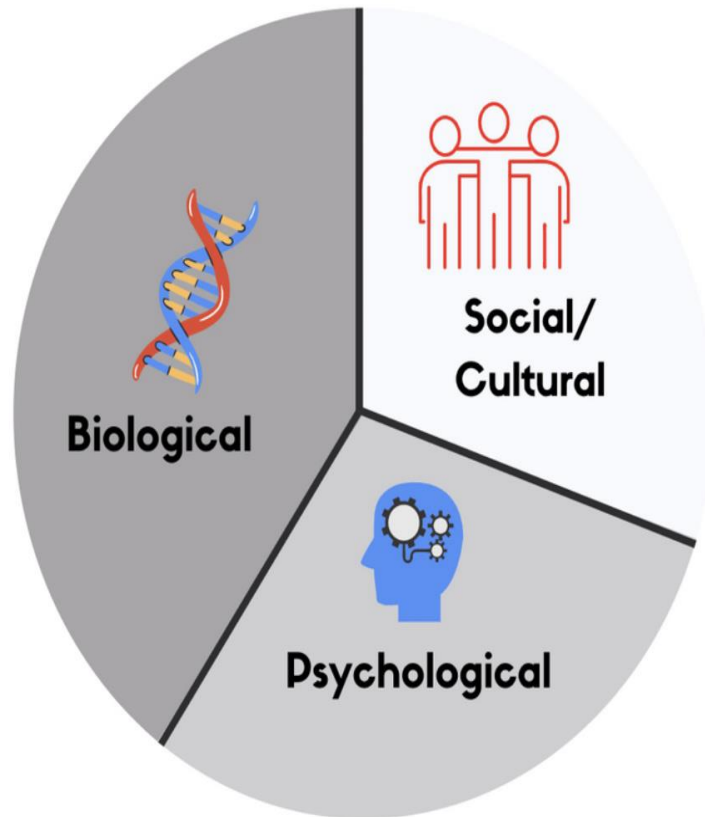
Subtype	Applicable to MDD?	Applicable to bipolar disorder?	Definition
Seasonal pattern	Yes	Yes	Episodes happen regularly at a particular time of the year
Rapid cycling	No	Yes	At least four episodes within the past year
Mood-congruent psychotic features	Yes	Yes	Delusions or hallucinations with themes that are consistent with the mood state (e.g., guilt, disease, or death themes accompanying depression)
Mood-incongruent psychotic features	Yes	Yes	Delusions or hallucinations with themes that do not match the valence of the depressive or manic episode
Mixed features	Yes	Yes	At least three manic symptoms are present during a depressive episode, or at least three depressive symptoms are present during a manic episode
Catatonia	Yes	Yes	Extreme physical immobility or excessive peculiar physical movement
Melancholic features	Yes	Yes (for depressive episodes)	Lack of pleasure in any activity, inability to gain relief from positive events, and at least three other symptoms of depression, such as a distinct quality of mood, depressive symptoms that are worse in the morning, waking at least 2 hours too early, loss of appetite/weight, psychomotor retardation or agitation, or guilt
Atypical features	Yes	Yes	Symptoms that are unusual for depressive or manic episodes are present
Peripartum onset	Yes	Yes	Onset during pregnancy or within 4 weeks postpartum
With anxious distress	Yes	Yes	At least two symptoms of anxiety are present
Suicide risk severity	Yes	Yes	Suicidal ideation, plans, or other risk indicators are present



Prevalence of depressive disorder

- 12-month prevalence of **Major depressive disorder** is around 7%. The prevalence is three times higher among younger age groups. Females outnumber males by around 2-3 times.
- 12-month prevalence of **Persistent Depression** is around 1%.
- 12-month prevalence of **premenstrual dysphoric disorder** among menstruating women is between 1.8% and 5.8%.

Etiology of mood disorders





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Biological factors

- Family and genetics
- Neurotransmitters
- Neuro-endocrine system
- Immune system
- Sleep dysfunction



Genetic

Overall genetics has greater role in bipolar disorders than unipolar disorders.

- Direct : by affecting neurotransmitters and HPA axis.
- Indirect: via modifying personality and cognition.

Risk of mood disorder is more common in people who have first degree relatives with the condition (bipolar>unipolar)



Neurotransmitters

- Neurotransmitters that play an important function in mood disorders are serotonin, dopamine and norepinephrine, which are decreased in episodes of depression.
- Serotonin is the neurotransmitter most commonly associated with depression.
- Dopamine is decreased in depression and increased in mania.



Hormonal

- Especially thyroid hormone dysregulation, increased TSH is associated with depression.
- Increase in HPA activity is associated with depression.



Psychological factors

- Personality traits of shyness, obsessive, and impulsive are predisposing traits of mood disorders, also cluster B and C personality disorders are risk factors for mood disorders.
- Attachment problems and early childhood adversity have been associated with depressive disorders.
- Certain personality disorders like borderline personality disorder and obsessive compulsive disorder OCD are more frequently associated with mood disorders.



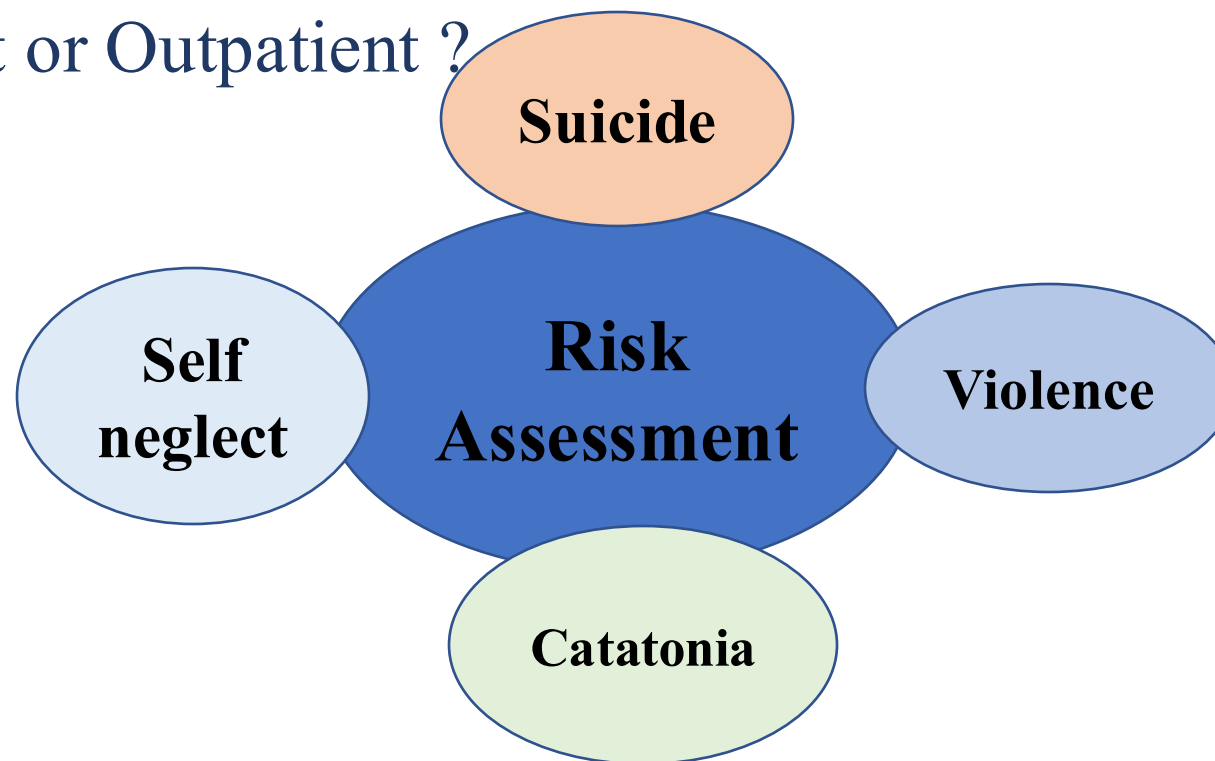
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Social factors

Stressful life events (like death of family members, loss of job,..etc.), traumatic events, and childhood abuse are major risk factors for development of mood disorders, especially in depressive disorders.

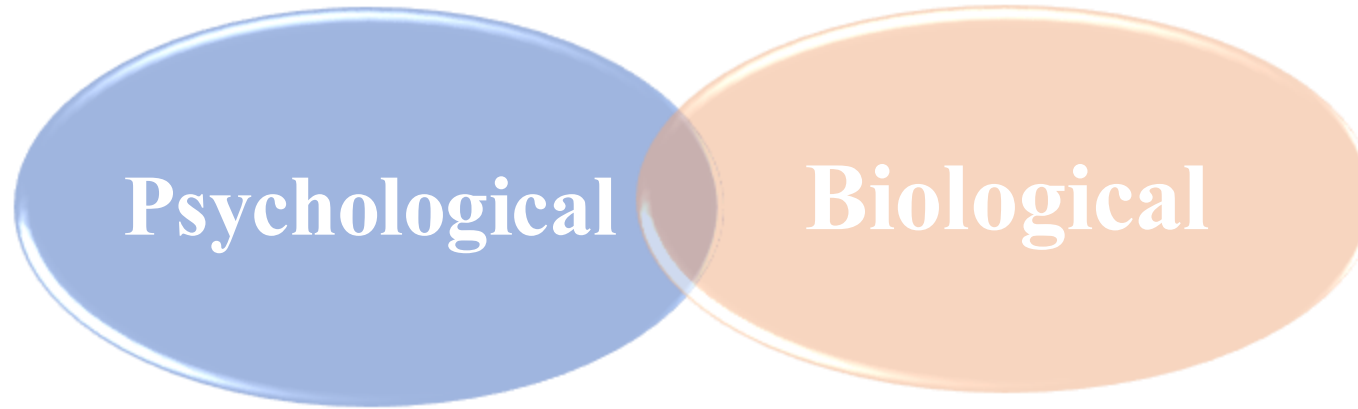
Treatment

Inpatient or Outpatient ?



Treatment

Bipolar Disorder



- Depends on the phase on the disease



Psychological

- As an adjunct to pharmacotherapy has been shown to be of significant benefit during the management of acute phase bipolar depression and maintenance phase of illness.
- Associated with reduced risk of relapse, better functioning and better treatment adherence.

Types

1. Psychoeducation
2. Interpersonal and social rhythm therapy (IPSRT)
3. Cognitive behavioral therapy (CBT)
4. Family focused intervention

Biological

- **Mood stabilizers**

Lithium, Sodium Valproate, Carbamazepine, Lamotrigine, etc.

- **Antidepressants**

SSRIs, TCAs, SNRIs, etc.

- **Antipsychotics**

1st Gen & 2nd Gen

- **Adjunctive medications**

Benzodiazepine, Anticonvulsants, Hypnotic-sedatives





Currently on treatment

- Stop antidepressant.
- Optimize the dose of the mood stabilizer which the patient is on.
- If the symptoms don't respond to the optimization of the dose or are severe, consider adding an antipsychotic medication.
- Benzodiazepines as per the need.

Currently not on treatment

1st line therapy

- Monotherapy: lithium, valproate, antipsychotic
- Combination: lithium + antipsychotics/valproate + antipsychotic
- Benzodiazepine as per the need.
- Depot antipsychotic may be considered if patient is not willing to take oral medications.



2nd line therapy

Monotherapy: change Lithium to Lamotrigine

Combination:

- Add antipsychotic if not used
- Change from Lithium+Antipsychotic to Valproate +Antipsychotic or vice versa
- Change lithium/Valproate to Carbamazepine
- Benzodiazepine on need
- ECT if not considered earlier
- Consider Clonazapine if mania is treatment refractory.

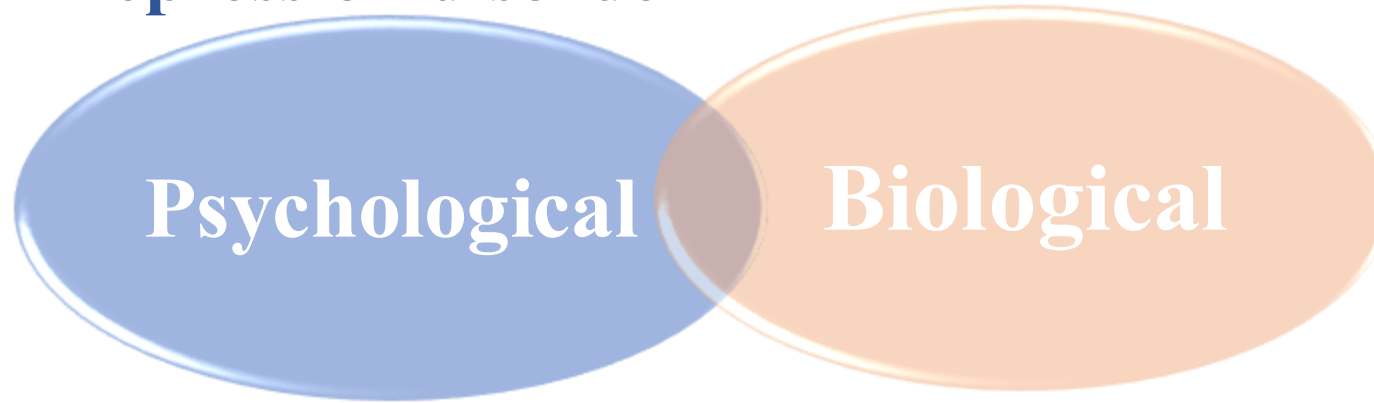


Maintenance treatment

- In terms of pharmacotherapeutic agents, best evidence for maintenance treatment is available for Lithium and Valproate.
- Olanzapine, Quetiapine (risk of metabolic side effects)
- Risperidone (prevention of depressive and manic episodes)
- Lamotrigine (prevention of depressive episodes)
- Aripiprazole, Ziprasidone and Asenapine (prevention of manic episode)
- Carbamazepine (less preferred compared to Lithium and Valproate because of its side effects and drug interactions)
- Psychotherapy

Treatment

Depression disorder



- Antidepressants
 - SSRIs (fluoxetine, citalopram, sertraline)
 - TCAs (Imipramine, nortriptyline)
 - SNRIs (Duloxetine)
 - MAOIs (tranylcipromine)
 - Others
- Augmentation (lithium, T3, T4, 2ND Gen antipsychotics)



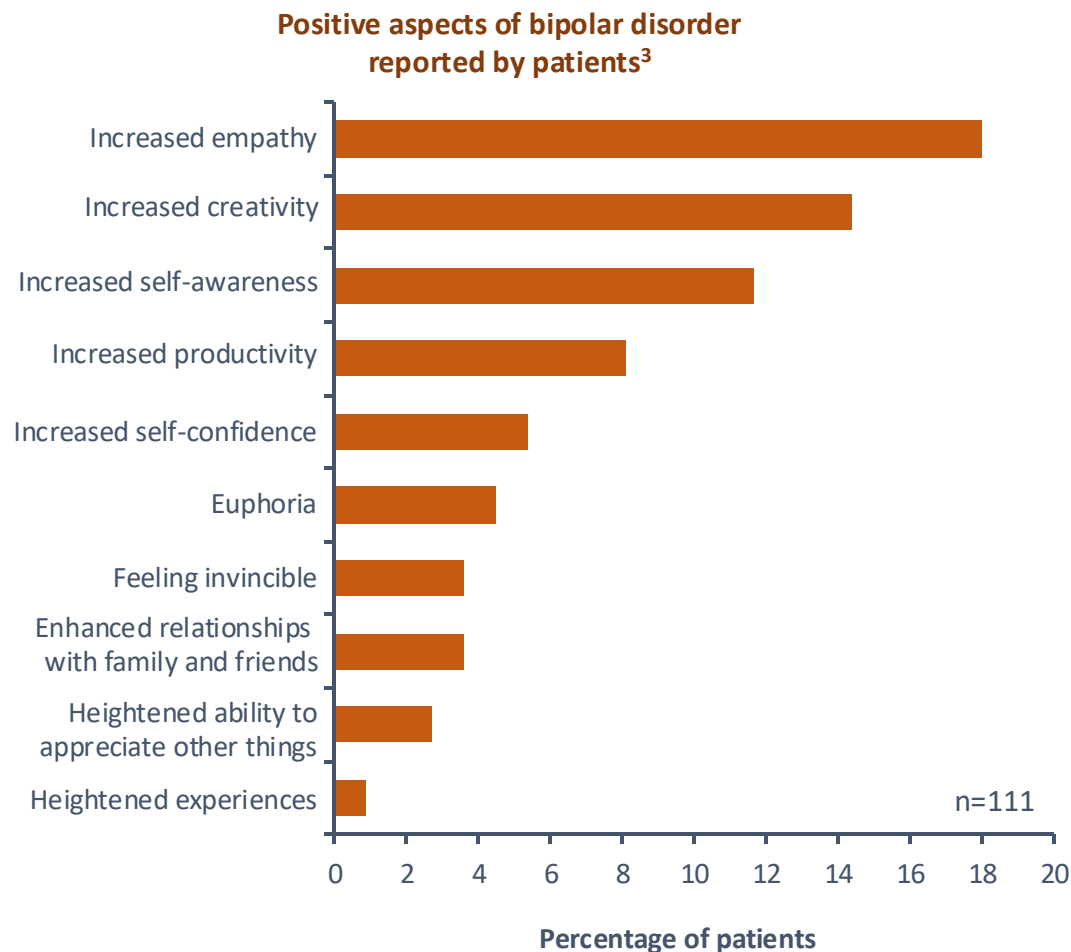
Indications

- Resistant Bipolar mania/depression
- Catatonic symptoms
- Need for rapid control of symptoms
- Presence of suicidal behavior which puts the life of the patient at risk
- Presence of severe agitation or violence which puts the life of others at risk
- History of good response in the past
- Augmentation of partial response to the pharmacotherapy
- Not able to tolerate pharmacotherapy
- Episodes of severe depression or mania during pregnancy



Strengths and positive characteristics

- The association between creativity and BD has been well-documented¹
- Other positive traits such as spirituality, realism, and resilience, can be observed in individuals with BD²
- In one study, 62% of patients with BD reported that there are advantages to their disorder³
 - Advantages were **three times** more likely to be reported for BD-II than BD-I³
- In another study, 48% of patients with BD reported that they would 'switch off' their BD, if the effect was reversible⁴
 - Some patients associate mania with 'enhanced qualities', and 'fun'⁴





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**Thank you
for Attention**